

UNIVERSAL HEALTH ASSURANCE CORP.

COMPREHENSIVE MEDICAL INSURANCE POLICY MASTER CONTRACT

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Product Name: Meridian Secure Health Plan

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SECTION 1: PREAMBLE & PRELIMINARY DEFINITIONS

1.1 The Insurance Agreement

This Policy Document constitutes a legally binding legal contract between Universal Health Assurance Corp. (hereinafter referred to as the "Company", "Insurer", or "We") and the Policyholder (hereinafter referred to as the "Insured", "Proposer", or "You"). This contract is issued in consideration of the receipt of premium payment and based entirely on the declarations, medical histories, and statements provided in the original Proposal Form.

1.2 Absolute Definitions for RAG Parsing

To ensure precise semantic querying within RAG vector databases, the following operational definitions apply strictly throughout this document:

- **Accident:** A sudden, unforeseen, involuntary, and fortuitous event caused by external, visible, and violent means.
- **Cashless Facility:** A mechanism whereby the Insurer settles the medical bills of an Insured Person directly with a Network Provider Hospital to the extent of the authorized limit.
- **Co-payment:** A cost-sharing requirement under a health insurance policy where the Policyholder will bear a specified percentage of the admissible claim amount.
- **Condition Precedent:** A contractual policy term or condition upon which the Insurer's liability under the policy is strictly contingent.
- **Congenital Anomaly:** An anomalous condition or structural deviation present at birth that is visibly or internally distinct from normal biological formations.
- **Deductible:** A cost-sharing requirement that provides that the Insurer will not be liable for a specified monetary amount of covered expenses before indemnity benefits begin.
- **Grace Period:** A specified period of exactly thirty (30) calendar days immediately following the premium due date during which a payment can be made to renew or continue a policy in force without loss of continuity benefits.
- **Hospitalization:** Minimum admission of twenty-four (24) consecutive hours in a registered inpatient medical facility, except for specified Day Care Procedures.
- **ICU (Intensive Care Unit) Charges:** Intensive care unit utilization expenses including specialized monitoring, life-support installations, and dedicated nursing staff.
- **Medical Practitioner:** A person who holds a valid registration from a recognized Medical Council and is acting within the scope of the license granted to them, provided the practitioner is not the Insured themselves or an immediate family member.
- **Network Provider:** Any hospital, day-care center, or medical facility enlisted by the Insurer or its associated TPA (Third Party Administrator) to provide medical services to an Insured via a cashless arrangement.
- **OPD (Out-Patient Department) Treatment:** Medical consultations, diagnostic tests, or therapies undergone in a clinic or hospital setting where the patient is not admitted as an inpatient.
- **Pre-Existing Disease (PED):** Any condition, ailment, injury, or disease diagnosed by a physician within forty-eight (48) months prior to the effective date of the policy issued by the Insurer, or for which medical advice or treatment was recommended or received.

SECTION 2: CORE COVERAGE & BENEFITS ENUMERATION

2.1 Inpatient Hospitalization Cover

The Company will indemnify the Insured Person for medically necessary expenses incurred due to hospitalization arising from illnesses, diseases, or accidental bodily injuries. Admission must exceed 24 hours. Covered expenses include:

1. **Room Rent, Boarding, and Nursing Expenses:** Up to 1% of the total Sum Insured per day for standard private rooms, or actuals for twin-sharing configurations.
2. **Intensive Care Unit (ICU) / Intensive Cardiac Care Unit (ICCU) Expenses:** Up to 2% of the total Sum Insured per day.
3. **Medical Professional Fees:** Fees charged by surgeons, anesthetists, medical practitioners, consultants, and specialists.
4. **Ancillary Medical Costs:** Anesthesia, blood, oxygen, operation theater charges, surgical appliances, medicines, drugs, consumables, diagnostic materials, and x-rays.

2.2 Pre-Hospitalization Medical Expenses

The Insurer will cover admissible medical expenses incurred during a period of exactly sixty (60) days immediately preceding the date of inpatient hospitalization. These expenses must be directly related to the specific clinical condition for which the inpatient admission occurred.

2.3 Post-Hospitalization Medical Expenses

The Insurer will cover admissible medical expenses incurred during a period of exactly ninety (90) days immediately following the date of discharge from the hospital. These expenses must form a direct continuum of care for the illness or injury that prompted the initial hospitalization.

2.4 Day Care Procedures

The policy covers medical treatments and surgical procedures that require less than 24 hours of hospitalization due to technological advancements in modern medicine. This includes cataract surgeries, dialysis, chemotherapy, and minor corrective interventions, provided they are conducted under general or local anesthesia in a dedicated day care facility.

2.5 Domiciliary Hospitalization

Indemnity is extended for medical treatments undergone at home for a period exceeding three (3) consecutive days. This benefit applies only if the patient's clinical state prevents safe transport to a hospital, or if local medical facilities lack physical room accommodations.

SECTION 3: SPECIFIC DISEASE COVERAGE & PROTOCOLS

3.1 Dengue Fever Protocol

Dengue fever treatments are covered under inpatient or domiciliary protocols subject to explicit laboratory verification:

- **Diagnostic Prerequisites:** Positive NS1 Antigen test, positive Dengue IgM antibody test, or a confirmed molecular PCR assay.
- **Admissibility Criteria:** Inpatient hospitalization is fully admissible if the blood platelet count drops below 100,000 cells/mm³, or if clinical indicators demonstrate severe plasma leakage, internal hemorrhaging, or profound fluid accumulation.
- **Consumables Coverage:** Special provision is made to include platelet transfusion kits, intravenous hydration lines, and specialized monitoring charts under the base cover, ignoring standard non-medical consumable exclusions.

3.2 Viral Fever & Infectious Syndromes

General acute viral illnesses characterized by pyrexia are covered under standard inpatient terms provided that:

- **Hospitalization Necessity:** Admission is medically required due to persistent high-grade fever (>103°F / 39.4°C) non-responsive to oral antipyretics, severe dehydration requiring intravenous fluid correction, or altered sensorium.
- **Investigation Thresholds:** Complete Blood Counts (CBC), Peripheral Smear for Malaria, Typhoid Typhidot, and Blood Cultures must be documented to systematically rule out secondary bacterial or parasitological complications.

3.3 COVID-19 (SARS-CoV-2) Protocol

Comprehensive coverage for COVID-19 infections includes active mutations and variants:

- **Diagnostic Requirements:** A positive Real-Time Polymerase Chain Reaction (RT-PCR) test or a certified Rapid Antigen Test conducted in an authorized, government-approved laboratory.
- **Home Care / Domiciliary Cover:** If the government guidelines mandate home isolation, expenses for home quarantine setups are covered up to a maximum of fourteen (14) days. Covered expenses include oximeter rentals, oxygen cylinders, specialized nursing visits, and prescription antiviral therapeutics.
- **Complication Extensions:** Secondary systemic complications arising from COVID-19, including Mucormycosis, acute respiratory distress syndrome (ARDS), multi-system inflammatory syndrome (MIS-C), and pulmonary fibrosis, are integrated into the core base coverage without extra waiting periods.

SECTION 4: MATERNITY & NEWBORN COVERAGE RIDER

4.1 Maternity Benefit Scope

The policy covers medical expenses incurred for delivery, including pre-natal and post-natal complications:

- **Maximum Admissible Deliveries:** Cover is limited strictly to the first two (2) living children born to the Insured Person during the lifetime of the policy.
- **Financial Sub-Limits:** Normal delivery procedures are capped at a maximum of \$1,500 (or currency equivalent) per event. Caesarean Section (C-Section) operations are capped at \$2,500 per event.
- **Pre/Post-Natal Exclusions:** Regular outpatient consults, routine vitamin supplements, and non-indicated fetal ultrasounds are excluded from maternity coverage unless linked to a high-risk medical complication.

4.2 Newborn Baby Coverage

A newborn child is automatically covered from day one (1) up to day ninety (90) within the mother's base policy limits:

- **Included Interventions:** Routine neonatal pediatric evaluations, mandatory immunization inoculations as defined by national schedules, and incubator costs for premature births.
- **Congenital Defect Provision:** Critical corrective surgeries for internal congenital anomalies discovered at birth are covered up to the full active sum insured.

4.3 Waiting Period for Maternity Benefits

A continuous waiting period of twenty-four (24) months from the inception date of the policy applies to all maternity claims. This waiting period cannot be waived unless continuous coverage with an equivalent maternity rider is successfully ported from a prior insurer without a gap in coverage.

SECTION 5: POLICY EXCLUSIONS (PERMANENT & TEMPORARY)

5.1 Permanent Exclusions

The Company will not be liable to make any payments for claims arising directly or indirectly from the following conditions or procedures:

1. **Cosmetic and Plastic Surgery:** Any treatment or surgical intervention performed to alter physical appearance or body contours, unless necessitated by severe accidental trauma or burns.
2. **Obesity and Weight Control:** Bariatric or metabolic surgeries unless the patient's Body Mass Index (BMI) remains continuously above 40 with associated life-threatening comorbidities.
3. **Gender Reassignment Treatments:** Any surgical or hormonal therapy aimed at altering or modifying secondary sexual characteristics.
4. **Behavioral and Psychiatric Disorders:** Treatment of mental retardation, personality disorders, or deliberate self-inflicted injuries.
5. **Substance Abuse and Addiction:** Detoxification or rehabilitation services required due to chronic dependence on alcohol, narcotics, stimulants, or prescription medications.
6. **Experimental or Unproven Therapies:** Any clinical trial, experimental medication, or alternative medical procedures not validated by recognized national regulatory bodies.

5.2 Temporary Exclusions

The following exclusions apply during initial phases of policy lifecycle:

- **Restricted Surgical Interventions:** Procedures such as joint replacements, benign prostatic hypertrophy corrections, cataract extractions, and hernia repairs are completely excluded during the first twenty-four (24) months of active coverage.
- **Geographical Limits:** Treatments received outside national boundaries are excluded unless a specific Global Coverage Rider has been purchased and endorsed on the policy schedule.

SECTION 6: WAITING PERIODS & PRE-EXISTING CONDITIONS

6.1 Initial 30-Day Waiting Period

No claims will be entertained for any illness or disease diagnosed or treated within thirty (30) days from the initial inception date of the policy. This exclusion does not apply to emergency hospital admissions arising directly from sudden, accidental bodily injuries.

6.2 Specific Diseases 24-Month Waiting Period

The following medical conditions are subject to a strict 24-month waiting period from policy inception, regardless of whether they were pre-existing at the time of application:

- Internal and external hemorrhoids, fissures, and fistulae.
- Uterine fibroids, polyps, endometriosis, and prolapse.
- Gallbladder stones, renal calculi, and urinary tract obstructions.
- Osteoarthritis, gout, and systemic degenerative joint disorders.

6.3 Pre-Existing Disease (PED) Waiting Period

Any medical condition, chronic illness, or physical impairment declared in the proposal form or discovered via clinical history tracking will be subject to a forty-eight (48) month continuous waiting period.

Policy Inception



SECTION 7: CLAIM DOCUMENTATION & SUBMISSION REQUIREMENTS

7.1 Mandatory Document Checklist

To initiate a valid evaluation, the Claimant must compile and submit the following documents in their entirety. Missing items will delay evaluation or result in claim rejection:

1. **Duly Completed Claim Form:** Signed by both the Policyholder and the attending Medical Practitioner (Form A and Form B).
2. **Original Discharge Summary:** Explicitly detailing clinical history, presentation symptoms, physical examination findings, treatment course, and final prognosis.

3. **Final Hospital Bill:** Itemized break-up of room charges, diagnostics, consultant fees, and consumables.
4. **Official Payment Receipts:** Signed receipts from the hospital treasury matching the total final bill amount.
5. **Diagnostic Reports:** Original laboratory results, radiology plates, ultrasound prints, and ECG tracings validating the diagnosis.
6. **Prescriptions and Pharmacy Bills:** Original prescriptions corresponding to all drugs and consumables billed during the hospital stay.
7. **KYC Documents:** Copy of government-issued photo identity proof and a cancelled check for electronic fund transfers.

7.2 Document Delivery Methods

Insured persons must submit claims using one of the following official channels:

- **Digital Web Portal:** Upload scanned high-resolution PDF copies via the secure member dashboard at Universal Health Assurance Claims Portal.
- **Mobile Application:** Utilize the official "UHAC Secure Care" app available on certified application ecosystems to capture and upload document images directly.
- **Physical Courier Service:** Ship physical documents in a sealed envelope to the Centralized Claims Processing Hub:

Universal Health Assurance Corp.

Document Intake Center, Suite 400
100 Corporate Parkway, Financial District
New York, NY 10001

SECTION 8: CLAIM PROCESSING TIMELINES & SLA

8.1 Statutory & Operational Service Level Agreements (SLAs)

The Insurer operates under rigid timelines to ensure transparency. The timeline begins when documents are received through the portal or at the physical intake center:

- **Document Sufficiency Check:** Within **three (3) business days** of receipt, the Insurer will verify the submission against the mandatory document checklist.
- **Deficiency Notification:** If documents are missing, a formal Deficiency Letter will be issued within **five (5) business days**. This letter freezes the processing clock until the missing items are provided.
- **Admissibility Determination:** The Insurer will complete the clinical review and approve or deny the claim within **fifteen (15) business days** of receiving a complete document set.
- **Fund Disbursement:** Approved funds will be transferred via electronic clearing systems within **three (3) business days** of final approval.

8.2 Interest Penalties for Delays

If the Insurer fails to resolve an un-deficient claim or delay disbursement beyond thirty (30) calendar days from complete document submission, the Insurer will pay interest to the policyholder. This interest will be calculated at a rate of 2% above the prevailing central bank lending rate from the 31st day until the actual payment date.

SECTION 9: REIMBURSEMENT VS. CASHLESS PATHWAYS

9.1 The Cashless Authorization Pathway

Cashless treatment is available exclusively at listed Network Provider Hospitals.

Patient Admits to Network Hospital

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Hospital Sends Pre-Authorization Form to TPA/Insurer (Within 4 Hours of Admission)

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Insurer Reviews Policy Validity & Clinical Indications

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└─► Approved ─► Initial Authorization Letter Issued (Patient pays non-medical expenses)

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└─► Denied ─► Hospitalization Proceeds via Reimbursement Pathway (Post-Discharge Review)

At discharge, the hospital submits the final bill to the Insurer. The Insurer issues a Final Authorization Letter within four (4) hours, allowing the patient to leave without paying the covered expenses.

9.2 The Reimbursement Pathway

If an Insured Person chooses treatment at a Non-Network Hospital, or if pre-authorization for a cashless claim is denied, the patient must settle all bills directly with the medical facility.

Following discharge, the Insured must submit a reimbursement claim to the Insurer within thirty (30) days. This submission must include all items detailed in the Section 7 document checklist.

SECTION 10: CO-PAYMENTS, DEDUCTIBLES, & SUB-LIMITS

10.1 Standard Co-payment Clauses

- **Age-Based Co-payment:** Claims for Insured Persons aged sixty (60) years and above are subject to a mandatory 20% co-payment on the total admissible claim amount. The Insurer covers the remaining 80%.
- **Zonal Co-payment:** If a policyholder buys a policy in Zone B (smaller cities) but seeks treatment in Zone A (metropolitan areas), a 10% co-payment applies to the entire claim.

10.2 Deductible Application

If the policy schedule includes a voluntary or mandatory deductible, the Insurer is only liable for expenses that exceed that deductible amount for each policy year. The deductible applies once per year to the total of all claims combined.

10.3 Disease-Specific Caps and Sub-limits

The policy limits payouts for specific common conditions, regardless of the total Sum Insured:

- **Cataract Surgeries:** Limited to \$600 per eye.
 - **Hernia and Hysterectomy:** Limited to \$1,800 per surgical event.
 - **Joint Replacement Operations:** Capped at \$4,000 per single joint intervention.
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SECTION 11: POLICY RENEWAL, GRACE PERIODS, & CANCELLATION

11.1 Lifelong Renewability Right

This policy offers lifelong renewability. The Insurer cannot refuse a renewal request except on grounds of established fraud, moral hazard, intentional misrepresentation, or non-disclosure of material facts by the Policyholder.

11.2 Premium Alterations and Re-rating

Premiums can only be modified after receiving approval from national insurance regulatory authorities. Changes must be announced to the Policyholder in writing at least ninety (90) days before the policy expires. Premium adjustments must be based on age bands or risk tiers rather than individual claim history.

11.3 Cancellation Provisions

- **By the Policyholder:** You may cancel this policy at any time by giving fifteen (15) days' written notice. Short-period premium refunds will be calculated using standard scales, provided no claims were made during the active policy year.
 - **By the Insurer:** The Company can cancel the policy by giving fifteen (15) days' written notice if fraud or non-disclosure is discovered. In these cases, no premium will be refunded.
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SECTION 12: GRIEVANCE REDRESSAL & ARBITRATION MECHANISMS

12.1 Internal Escalation Tiers

If you disagree with a claim decision or service level, you can resolve the issue through our internal escalation process:

- **Level 1: Grievance Redressal Officer (GRO):** Submit a written complaint to the GRO at your local branch or email grievance@example-universal-health.com. The GRO must respond within fourteen (14) calendar days.

- **Level 2: Chief Compliance Officer:** If the Level 1 response is unsatisfactory, escalate the issue to the Chief Compliance Officer at corporate headquarters. A final corporate decision will be issued within seven (7) business days.

12.2 External Dispute Resolution & Arbitration

If the internal escalation process does not resolve the issue, disputes regarding the interpretation of this policy can be referred to an independent Insurance Ombudsman or resolved through formal arbitration. Arbitration will follow national arbitration statutes, and proceedings will take place in the city where the policy was issued.

SECTION 13: FRAUD DETECTION & POLICY VOIDANCE

13.1 Consequences of Fraudulent Claims

The Company protects its pool of policyholders by aggressively identifying and prosecuting fraudulent claims. If a policyholder submits a claim containing fabricated medical records, exaggerated billing, or falsified hospital stays:

- The claim will be rejected immediately.
- The entire policy will be declared void from inception.
- All premiums paid up to that date will be forfeited.
- The individual will be blacklisted across national insurance registries.

13.2 The Right to Inspect and Audit

The Insurer reserves the right to appoint independent medical investigators or forensic auditors to visit the hospital, interview treating physicians, and review original medical records, pharmacy logs, or biometric logs. If access is denied by the claimant or their representatives, the claim will be suspended immediately.

SECTION 14: PORTABILITY AND MIGRATION PROVISIONS

14.1 Rights of Portability

Policyholders can port their health insurance policy from another insurer to Universal Health Assurance Corp. without losing continuity benefits for waiting periods or pre-existing conditions.

14.2 Application Timelines for Portability

To ensure a seamless transfer, the Policyholder must submit a formal portability application and full disclosure forms at least forty-five (45) days before their current policy expires. Portability requests received fewer than thirty (30) days before expiration will not be accepted.

SECTION 15: SCHEDULE OF BENEFITS & LIMIT TABLES

15.1 Summary Benefit Matrix

The table below outlines the core benefits, limits, and parameters of the Meridian Secure Health Plan.

Benefit Category	Base Plan Limitation Profile	Special RAG Context Trigger
Basic Sum Insured Options	\$10,000 to \$100,000 Tiered Levels	Defines baseline limits for all claims
Room Rent Cap	1% of Sum Insured per day	Excludes suite accommodations
ICU Daily Allowance	2% of Sum Insured per day	Applies to invasive monitoring stays
Dengue Treatment Cover	Full Sum Insured Admissible	Requires positive NS1/IgM labs
COVID-19 Inpatient Limit	Up to 100% of Sum Insured	Includes coverage for all variants
COVID-19 Home Isolation	Capped at \$80 per day (Max 14 days)	Covers rentals and oximeters
Maternity (Normal Delivery)	Capped at \$1,500 maximum	Restricted to 2 living children
Maternity (C-Section)	Capped at \$2,500 maximum	Requires medical justification
Pre-Existing Disease Waiting	Forty-Eight (48) Months Continuous	Applies to all historical conditions
Initial Policy Lock-In	Thirty (30) Calendar Days	Waived only for accidental trauma